

New Generation English B· Diagnostic Test

C1 (Advanced Academic) B· Full Test

READING

The paradox of expertise: why specialists often see less

There is a particular irony in advanced professional knowledge: the more an expert knows about a narrow subject, the more they begin to see only that subject. A radiologist who has reviewed ten thousand chest X-rays develops an exceptional eye for chest pathology and a measurable blind spot for anomalies in other parts of the same image. A senior litigator who has won hundreds of cases on procedural grounds will, even unconsciously, frame new disputes in procedural terms. The expertise produces depth at the cost of width.

This is not a personal failing. It is a structural feature of how human attention works at high levels of specialisation. The brain, presented with familiar patterns, recognises them faster than it questions them. That speed is precisely what makes an expert useful in their domain. But the same speed becomes a liability the moment the problem in front of them is not exactly the one their experience predicts.

The practical lesson is not that expertise is unreliable. It clearly is reliable, within its proper domain. The lesson is about how expert advice should be requested and how it should be received. A wise client does not ask a single specialist to interpret a complicated situation; they ask two or three, ideally from neighbouring fields. A wise expert, in turn, names the limits of their own perspective without apology, treating those limits as professional information rather than weakness.

The deeper, less comfortable implication concerns the institutions that train experts. Most professional education rewards the deepening of a chosen field, not the cultivation of awareness about its blind spots. Doctors are not formally trained in the psychology of medical error. Lawyers are not formally trained in the limits of their own argumentation styles. Where such training does exist, it is usually optional, marginal and easy to ignore. As a result, the paradox of expertise reproduces itself, generation after generation, in fields that genuinely need it discussed.

1. What "particular irony" does the writer identify in advanced professional knowledge?

A) The more an expert knows about a narrow subject, the more they begin to see only that subject.

B) Experts are always wrong.

C) Knowledge has no link with attention.

2. What does the radiologist example illustrate?

A) A blind spot for anomalies outside the chest, despite extreme skill within it.

B) Perfect general vision.

C) An unrelated medical phenomenon.

3. How does the brain handle familiar patterns?

A) It refuses to recognise them.

B) It recognises them faster than it questions them.

C) It rejects them automatically.

4. When does an expert's speed become a liability?

A) The moment the problem in front of them is not exactly the one their experience predicts.

B) When the expert is tired.

C) When the case is too easy.

5. What does a wise client do?

A) Trusts a single specialist completely.

B) Asks two or three specialists, ideally from neighbouring fields.

C) Avoids specialists altogether.

6. What does a wise expert do?

A) Pretends to know everything.

B) Names the limits of their own perspective without apology.

C) Refuses to discuss limits.

7. What is missing from doctors' formal training, according to the text?

A) Basic anatomy.

B) The psychology of medical error.

C) Clinical examination.

8. What is the deeper implication of the article?

A) The paradox of expertise reproduces itself, generation after generation.

B) Expertise will disappear soon.

C) Training programs are perfect as they are.

LISTENING

Lecture: the diagnostic temptation in social commentary

Script:

In today's lecture I want to focus on what I will call the diagnostic temptation in social commentary. The case I will use is the recent discussion of loneliness as an epidemic.

The vocabulary of epidemic is unusually powerful. It moves a problem from the realm of personal misfortune into the realm of public health, which means budgets, agencies and policy. That is often useful. The 2023 Surgeon General advisory in the United States, for instance, would have attracted very little attention if it had used softer language. By choosing the word epidemic, it secured a level of seriousness that purely sociological framings rarely achieve.

However, the same vocabulary can mislead. An epidemic implies a pathogen, a vector and individual cases. Loneliness, when examined carefully, behaves more like an ecological condition than a contagion. It emerges from the way cities are built, from how working hours have shifted, from which institutions have been allowed to weaken. It is not transmitted; it is produced.

This distinction matters because the two framings recommend very different interventions. A pathological framing favours individualised responses: therapy, medication, perhaps an app. An ecological framing favours structural responses: walkable neighbourhoods, public libraries, time-protective labour laws. Both can coexist, but they are not equivalent.

The deeper question — and this is where I want you to push beyond conventional readings — is who benefits from each framing. Clinical and technological interventions are easier to monetise and easier to study. Structural interventions are slower, less visible and politically uncomfortable, because they imply that current city design, current labour culture, and current public spending priorities have been quietly producing the very condition they now claim to treat.

I am not suggesting that individual treatment is useless. I am suggesting that, taken in isolation, it acts as a kind of moral comfort. It allows societies to be seen to be doing something about loneliness without changing the structures that produce it. That, I would argue, is precisely the conservatism of a diagnostic vocabulary applied to a structural problem.

1. What does the lecturer call the diagnostic temptation?

A) A tendency to treat structural problems as medical conditions.

B) A reluctance to use scientific terms.

C) An unwillingness to consult patients.

2. Why does the lecturer say the vocabulary of epidemic is powerful?

A) It moves a problem from personal misfortune into public health, with budgets and agencies.

B) It sounds friendly.

C) It is easy for journalists to misunderstand.

3. How does the lecturer say loneliness actually behaves?

A) Like a contagion.

B) More like an ecological condition than a contagion.

C) Like a hereditary disease.

4. What does a pathological framing favour?

A) Individualised responses such as therapy, medication, or an app.

B) Restructuring labour laws.

C) Closing public libraries.

5. What does an ecological framing favour?

A) Walkable neighbourhoods, public libraries, time-protective labour laws.

B) Banning social media.

C) Higher taxes on therapists.

6. Why are clinical and technological interventions easier?

A) They are easier to monetise and easier to study.

B) They cost nothing.

C) They are politically uncontroversial in every country.

7. Why does the lecturer say structural interventions are politically uncomfortable?

A) Because they imply that current city design and labour culture have been producing the very condition they now claim to treat.

B) Because they require imported expertise.

C) Because they only work in small towns.

8. What is the lecturer's main warning about individual treatment in isolation?

A) That it acts as a kind of moral comfort — appearing to do something without changing the structures.

B) That it is harmful in every case.

C) That it should be banned.

WRITING

Write 240-300 words. The epidemic of loneliness has become a major theme in the public discourse of wealthy societies. To what extent should the response be clinical (therapy, medication, apps) and to what extent structural (urban design, public spaces, working hours)? Discuss with examples and qualify your position carefully.

SPEAKING

Record 3 minutes. Explain why the framing of a social problem often matters as much as the problem itself. Use loneliness as your central example. Include one historical or international comparison and one limitation of your own argument.

